

ETHIOPIAN HEALTH PROFESSIONALS LICENSING EXAMINATION(EHPLE) PRACTICE TEST BOOKLET 2025

PSYCHIATRIC NURSING



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MINISTRY OF HEALTH-ETHIOPIA

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HEALTHIER CITIZENS FOR PROSPEROUS NATION!



Message from the State Minister, Ministry of Health – Ethiopia



Mrs. Frehiwot Abebe

Improving the quality of healthcare remains a critical global priority and is essential for achieving sustainable development and universal health coverage. One of the most effective ways to ensure high standards in healthcare delivery is through the consistent assessment of professional competencies. In line with this commitment, the Ministry of Health of Ethiopia launched the Ethiopian Health Professionals Licensing Examination (EHPLE) in 2019 for undergraduates in seven core health disciplines. Since its inception, the program has expanded to encompass 17 health professions, reflecting our continued effort to build a competent and responsive health workforce.

The EHPLE was established with the primary aim of identifying and certifying health professionals who possess the minimum competencies required to provide safe and effective care. This ensures that all practicing professionals are equipped to meet the expectations of the communities they serve. The exam is coordinated by the Health and Health-Related Institutions and Professionals' Regulatory Lead Executive Office (LEO), which consists of four specialized desks. The LEO plays a vital role in system strengthening, enabling the exam to be conducted with increased efficiency, organization, and quality assurance.

It is important to recognize that the EHPLE is not a conventional academic or recruitment examination. Rather, it is a rigorous, competency-based assessment designed to evaluate the practical skills and critical thinking abilities that are essential for professional practice. This *Practice Items Booklet* is part of our broader strategy to support examinees and educators in preparing for the licensing process. It offers a realistic overview of the exam structure, promotes a deeper understanding of its objectives, and fosters transparency and sustainability in implementation.

The development of this booklet has been made possible through the collaborative efforts of experts from higher education institutions, the Addis Ababa University Testing Center, and professional associations. Their insights and dedication have been instrumental in shaping this resource. I extend my heartfelt thanks to all contributors and to the Ministry of Health team for their tireless work and steadfast commitment to excellence in healthcare regulation.

We believe this booklet will serve as a valuable tool for candidates preparing for the examination and for educators guiding them toward success. Together, we are building a stronger, more competent health workforce for Ethiopia.

Introduction EHPLE

Welcome to the 2025 Edition of the Ethiopian Health Professionals Licensing Examination Practice Test Booklet.

This resource is designed to support candidates in their preparation for the national licensing examination, ensuring they are well-equipped with the knowledge and clinical reasoning skills required to deliver competent and ethical healthcare across Ethiopia.

In Ethiopia, the licensing examination serves as a pivotal tool in evaluating the competency of health professionals who have completed their academic training and are ready to transition into independent practice. The exam not only ensures public safety by verifying professional readiness but also upholds national and international standards in healthcare service delivery. As the healthcare system continues to expand and evolve, a strong, well-regulated professional workforce becomes even more essential.

This practice booklet contains 200 multiple-choice questions modelled after the structure, content, and format of the actual licensing examination. The questions span across core clinical domains essential for entry-level practice. This includes areas such as Patient care, leadership and management, ethics, and Health Promotion as outlined in the Information Booklet 2024.

The booklet is intended to:

- Familiarize candidates with the structure and cognitive level of the actual licensing examination.
- Offer opportunities for self-assessment and identification of strengths and gaps in knowledge.
- Encourage critical thinking and clinical decision-making in scenarios reflective of real-world healthcare practice in Ethiopia.
- Reduce exam-related anxiety by providing realistic practice experiences.

Administered under the auspices of the Ministry of Health and relevant regulatory bodies, the Ethiopian Health Professionals Licensing Examination is grounded in evidence-based frameworks and aligned with both national needs and global best practices. It has been structured to assess a range of competencies including medical knowledge, communication, professionalism, and safe clinical practice.

This practice test is not a predictive tool, and performance on it does not guarantee similar outcomes on the actual exam. However, it provides an invaluable opportunity for familiarization with question styles, pacing strategies, and content review. We strongly recommend that candidates simulate real testing conditions while using this resource, including timing their responses and reviewing rationale for both correct and incorrect answers.

The goal of licensing examinations is not only to regulate entry into the profession but also to inspire excellence and lifelong learning. By engaging with this practice test, you are taking a proactive step toward professional development, accountability, and a commitment to serving your patients with competence and compassion.

We wish you success on your licensing journey—and beyond.

Acknowledgements

This *Practice Test Items Booklet for the Ethiopian Health Professionals Licensing Examination* is the result of a collaborative effort among dedicated professionals committed to advancing the quality of health sciences education in Ethiopia. The Ministry of Health extends its sincere appreciation to all those who contributed their time, expertise, and insights to the development of this essential resource.

Special recognition is given to the item developers and reviewers from various higher education institutions—both public and private—whose academic rigor and practical experience were instrumental in shaping the content and ensuring its relevance to real-world healthcare practice. Their contributions have helped create a high-quality, standardized practice tool that reflects national competencies and global benchmarks.

We are especially grateful to the team at the Addis Ababa University Testing Center, whose technical support and academic guidance were critical throughout the process. Their leadership in assessment design and educational evaluation has significantly strengthened the quality of this booklet.

We also thank the committed staff of the Ministry of Health, particularly those within the Health and Health-Related Institutions and Professionals' Regulatory Lead Executive Office (HHRIPR LEOs), whose coordination, oversight, and unwavering dedication ensured the successful completion of this project.

Together, these collective efforts reflect our shared vision of building a competent, ethical, and resilient health workforce for Ethiopia.

Psychiatric Nursing

1. A 10-year-old boy has come to a psychiatry OPD with complaints of difficulty in maintaining focus in school, talkativeness, interrupting teachers while teaching, often losing his pencil, forgetting where he puts his books, and failing to read instructions during exams, which leads him to get a low score. At home, he does not seem to listen to what his parents tell him and is reluctant to do his homework. During psychiatric evaluation, he is easily distracted, irritable, tapping his hands and nodding his head repeatedly. This problem has continued since he was 4 years old. Otherwise, he has no other medical disorders and is not on medication.

What is the most likely diagnosis in this case?

- A. Intellectual disability disorder
- B. Attention deficit hyperactivity disorder
- C. Disruptive mood dysregulation disorder
- D. Oppositional defiant disorder

ANSWER: B

2. A 5-year-old girl who lost her mother by accident has persistent and excessive worry. She is reluctant to go to school and bed without. While sleeping, she experiences terrifying dreams, which she recalls as seeing her caregiver leave her and go to another city. Most of the time, she complains of headaches, nausea, and stomach aches when the nanny goes shopping. The duration of the symptoms is 4 weeks.

What is the most likely diagnosis in this case?

- E. Separation anxiety disorder
- F. Performance anxiety disorder
- G. Generalized anxiety disorder
- H. Agoraphobia

ANSWER: A

3. A paediatrician referred a 12-year-old girl with a history of urine incontinence to a psychiatry OPD for further evaluation. The child had achieved continence when she was 4 years old, but later she was found wetting her bed and clothes. She was on imipramine for a month. She complained of polydipsia, polyuria, and severe headache associated with sleep disturbance. Her medical history showed that she had type 1 diabetes mellitus and generalized absent seizures.

What is the LEAST LIKELY risk factor causing incontinency in the above case?

- A. Sleep disturbance
- B. Diabetes mellitus
- C. Absent seizure
- D. Severe headache

ANSWER: D

4. A group of psychiatrists visited a child mental health institution and provided free medical services. They had done a standard test to screen children for intellectual disability.

What is the most likely risk factor in this case?

- A. Age
- B. Genetics
- C. Being female
- D. Sexual abuse

ANSWER: B

5. A third-grade girl has difficulty in social-emotional reciprocity and in following rules of conversation, stereotyped motor movement, deficient in the conceptual, social, and practical domains beginning at four years. The deficit has affected her academic performance and peer relationships.

What is the LEAST LIKELY differential diagnosis based on the above information?

- A. Separation anxiety disorder
- B. Autism spectrum disorder
- C. Social communication disorder
- D. Intellectual disability disorder

ANSWER: A

6. A 13-year-old girl has come to a psychiatry outpatient department with complaints of being unable to stay still and shouting. On examination, she looks agitated, answers questions while the clinician asks her, and cannot stay quiet. She is well-groomed, cooperative, but restless, and looks anxious. These symptoms started 8 months ago.

What is the most likely diagnosis in this case?

- A. Attention deficit hyperactivity disorder
- B. Disruptive mood dysregulation disorder
- C. Generalized anxiety disorder
- D. Autism spectrum disorder

ANSWER: A

7. A 12-year-old child with a history of nocturnal and diurnal enuresis associated with maintenance insomnia was initially prescribed Imipramine 25mg po noct. Little improvement was seen after adequate treatment duration, and a clinician added 12.5mg PO morning.

What is the most likely anticholinergic adverse effect related to this drug?

- A. Blurred vision
- B. Dizziness
- C. Cardiac arrhythmia
- D. Hypotension

ANSWER: A

8. A 12-year-old girl is referred to a psychiatry unit with complaints of difficulty in staying focused, talkativeness, inability to wait once turn, and problem of forgetting where she puts her school materials and fails to read instructions during exams, which leads her to get bad results, and carelessly crossing the road.

What is the most likely differential diagnosis in the above case?

- A. Mild neurocognitive disorder
- B. Attention-deficit hyperactivity disorder
- C. Major depressive disorder
- D. Disruptive mood dysregulation disorder

ANSWER: B

9. A 7-year-old female child with a history of mild intellectual disability disorder visited a hospital for a general medical check-up. Her medical history, physical examination, and laboratory investigation showed urinary tract infection, chronically holding bowel movement, ineffective sphincter control, and abnormal innervation of the anorectic region, and diagnosed with encopresis and has a family history of similar disorder.

What is the most likely risk factor in the above case scenario?

- A. Genetics
- B. Age
- C. Chronic diarrhoea
- D. Diabetes mellitus

ANSWER: A

10. A 6-year-old girl was referred to a child psychiatry unit because she refused to sleep alone, and nearly every morning, she was crying and exhibiting violent tantrums, not to go to kindergarten. When a psychiatrist asked her why she cried and did not want to go to kindergarten, she replied that "something bad would happen to mommy if I went to school". The psychiatry Nurse diagnosed her with separation anxiety disorder.

What is the most appropriate psychotherapy in the above case?

- A. Psychodynamic psychotherapy
- B. Cognitive behavioural therapy
- C. Psychoanalytical psychotherapy
- D. Interpersonal therapy

ANSWER: B

11. A child with intellectual disability has been admitted to a psychiatry inpatient unit for evaluation. In the ward, a nurse is assessing for risk of possible injury as a result of environmental conditions interacting with the individual's adaptive and defensive resources.

What is the most likely appropriate nursing intervention to ensure client safety?

- A. Creating a safe environment for the client
- B. Remaining with the client during initial interactions with others
- C. Use simple language to explain acceptable and unacceptable behaviours
- D. Maintain consistency of staff assignment over time

ANSWER: A

12. A paediatrician has assessed a 5-year-old child with complaints of delay in language development and quite limited play skills as compared to his siblings. He is resistant to changes in routine and unusually sensitive to aspects of the inanimate environment. He uses his toys in unusual and idiosyncratic ways. The mother was normal during pregnancy, labour, and delivery of the child, and there was no family history of neurodevelopmental disorder or other major psychiatric conditions.

What is the most likely prognostic factor for the above case?

- A. Support
- B. Abnormal pregnancy
- C. sexual abuse
- D. delay in treatment

ANSWER: A

13. A 55-year-old man was brought to a psychiatry clinic by his son for the sudden onset of being physically aggressive and talking to himself for two weeks. He usually fails to sleep at night due to the fear of the frightening image of unknown objects. On examination, a psychiatry nurse noticed that he is able to reply to 'serial three' and has a good level of awareness. Other mental disorders and substance use problems were not observed.

What is the most relevant history needed before reaching a diagnosis?

- A. Assess for medical cause
- B. Assess for cognitive deficit
- C. Assess for emotional problems
- D. Assess the impact it caused

ANSWER: A

14. A female client came to a psychiatry clinic with complaints of a cutting sensation in her brain and hearing of voice from an unknown source of two years duration. During the interview, she speaks new words, repeats the same answer for different questions, and shows a low facial expression. A psychiatric nurse considers schizophrenia in her diagnosis.

What is the relevant information needed to confirm the diagnosis?

- A. Presence of an additional symptom of delusion
- B. Presence of functional impairment
- C. Presence of additional negative symptoms
- D. Presence of a family history of the illness

ANSWER: B

15. A patient was brought to a psychiatry OPD by his family with complaints of recurrent urges to go out, suspiciousness, and loss of personal care for the last two years. When asked for the problem, he replied that "I am healthy, I do not know why they brought me here".

What is the most appropriate interpretation of the patient's response?

- A. His insight was poor
- B. His abstract thinking is impaired
- C. His general knowledge is poor
- D. His judgment is impaired

ANSWER: A

16. A 17-year-old high school student has developed a behavioural change of one year duration. He thinks that his idea is taken away by the media, and he suspects unidentified flying objects of sending him a message. In the interview, he goes around without answering directly for questions. His identical twin has also started to show similar behavioural changes.

What is the risk of getting the problem from the latter identical twin?

- A. 12 percent
- B. 17 percent
- C. 40 percent
- D. 50 percent

ANSWER: D

17. A patient with a diagnosis of brief psychotic disorder has been brought to a psychiatry unit for lack of improvement. He has taken an adequate dose of haloperidol for two months. Despite this, he still has episodes of behavioural disturbance followed by complete recovery.

What is the most likely history needed to confirm the diagnosis of this patient?

- A. History of substance
- B. Amnesia of the behavioural disturbance
- C. History of sexual abuse
- D. Family history of substances

ANSWER: B

18. A 26-year-old female patient was recently admitted to a psychiatric ward with complaints of physical aggression and loss of care for herself. She sometimes laughs out loud for no reason. At the same time she becomes retarded, depressed and being tearful. Laboratory investigation has revealed a low level of T3 and T4.

What is the most likely interpretation of the laboratory finding?

- A. The laboratory finding has nothing to do with the treatment plan
- B. Her mood symptom is caused by abnormal T3 and T4
- C. Her psychotic symptom is caused by abnormal T3 and T4
- D. The abnormal laboratory indicates a fair prognosis

ANSWER: B

19. A 26-year-old man has been brought to a psychiatry emergency unit with a history of five months of being suspicious and destroying household materials. He refuses to take food from his wife for fear of being poisoned. During the interview, he smiles without reason but moves his body as if he is dancing.

What is the most likely diagnosis in this case?

- A. Bipolar I disorder
- B. Schizoaffective disorder
- C. Schizophrenia
- D. Schizophreniform Disorder

ANSWER: D

20. A 34-year-old woman came to a psychiatry unit with a complaint of abnormal behaviour such as social withdrawal, depressed mood, agitation, restlessness, and mumbling that lasted seven months. On mental status examination, the psychiatry nurse on duty identified waxy flexibility, echolalia, and echopraxia. Vital signs showed Blood Pressure of 120/80 mmHg, Pulse rate of 110 beats/min, and Temperature of 38.5 degrees centigrade.

What is the most likely diagnosis for this patient?

- A. Catatonia secondary to schizophrenia
- B. Catatonia secondary to major depressive disorder
- C. Major depressive disorder
- D. Schizophrenia

ANSWER: A

21. A man was brought to a psychiatry emergency unit by police after breaking the car of a diplomat. On arrival, he was poorly dressed and preferred to sit on the floor rather than sit in a chair. He seemed suspicious as he was looking around and sometimes talking to himself. He could not reply to a clinician, and there was no caregiver with him. The clinician has decided to put a provisional diagnosis until sufficient information is obtained.

What is the most likely diagnosis based on the presentation?

- A. Unspecified schizophrenia spectrum and other psychotic disorder
- B. Other specified schizophrenia spectrum and other psychotic disorder
- C. Schizophrenia, disorganized type
- D. Bipolar I disorder, currently manic with psychotic features

ANSWER: A

22. A 42-year-old patient was brought to an emergency department by police in handcuffs. He was speaking like "Shut up! I told you I do not want to do it!" He was also aggressive and had been looking argumentative for the last three months. The first episode occurred two years ago and lasted only about four months. There was no history of substance use or medical problems.

What is the most likely diagnosis for this patient?

- A. Schizoaffective disorder
- B. Schizophreniform Disorder
- C. Brief psychotic disorder
- D. Schizophrenia

ANSWER: D

23. A patient was admitted to a medical ward for a diagnosis of hepatic encephalopathy a month ago. Recently, he was found to be mute, opposing to the instructions of a nurse, showing facial grimacing and sometimes becoming physically aggressive for no reason. Other psychiatric manifestations were not found.

What is the most likely diagnosis for this patient?

- A. Psychotic disorder due to hepatic encephalopathy
- B. Medication-induced psychotic disorder
- C. Catatonic disorder due to hepatic encephalopathy
- D. Delirium due to hepatic encephalopathy

ANSWER: C

24. A 42-year-old woman had become increasingly convinced that tiny bugs had burrowed underneath her skin. She tried to rid herself of them by washing multiple times each day with medicated soap and shampoo. She visited several dermatologists, but the problem was not evident. Consequently, she decided to asphyxiate the bugs by covering her body with gasoline and holding it against her skin with plastic wrap. It is now about two months since the onset of the problem.

What is the most likely diagnosis in this case?

- A. Schizophreniform disorder
- B. Illness anxiety disorder
- C. Delusional disorder
- D. Obsessive-compulsive disorder

ANSWER: C

25. A male client was brought to a psychiatry clinic by his mother with complaints of talking alone for no reason and providing irrelevant responses in conversation. During an interview, the patient replied with similar verbal responses to different unrelated questions.
What is the most appropriate interpretation of the patient's response?
- A. Perseveration
 - B. Tangentiality
 - C. Derailment
 - D. Verbigeration
- ANSWER: A
26. A 38-year-old male client who lost some of his property has been brought to a psychiatric clinic by his father with complaints of an urge to go out and hearing voices without an external stimulus and suspiciousness of six months duration. His twin brother is suffering from schizophrenia. He occasionally uses different kinds of substances.
What is the most likely cause of the symptoms in this case?
- A. History of recent loss
 - B. His age
 - C. History of substance use
 - D. Genetic predisposition
- ANSWER: D
27. A patient with a diagnosis of schizophrenia attributes her illness to her divorce that occurred two months before the onset. She smokes cigarettes daily. In her personal history, she was sexually abused by her father-in-law during her early years. The history further revealed a prolonged prodromal phase of schizophrenia.
What is the most likely predisposing risk factor for this patient?
- A. Being sexually abused
 - B. Being divorced
 - C. Being a user of cigarettes
 - D. Being Female
- ANSWER: A
28. A female patient comes to a psychiatry clinic with agitation, crying without apparent reason, and confusion. Before the onset of the current symptoms, the patient was diagnosed with delusional disorder and put on fluphenazine. Further investigation revealed muscle rigidity, high temperature, and increased creatinine phosphokinase.
What is the most likely appropriate management of this patient?
- A. Add fluoxetine
 - B. Add small dose of benzodiazepine
 - C. Immediately discontinue the medication
 - D. Give the medication every other day
- ANSWER: C

29. A 65-year-old woman was brought to a psychiatry hospital with talking and laughing alone for the last six months. She believed that she had committed an unforgivable sin and deserved death. On mental status examination, downcast gaze, tearfulness, impaired concentration, and mood-congruent delusion were identified. There was no history of substance use.

What is the most likely differential diagnosis of this patient?

- A. Delirium
- B. Major neuro-cognitive disorder, with behavioural disturbance
- C. Schizophreniform disorder, with good prognostic features
- D. Major depressive disorder with psychotic features

ANSWER: D

30. A 55-year-old woman has been brought to a psychiatry clinic for experiencing behavioural change of collecting dirty materials for a year's duration. She cannot take a shower by herself unless her children do so. She is reckless and refuses to obey any instructions. During the interview, she carried a bag filled with pieces of material of no value.

What is the most likely psychiatric nursing diagnosis of this patient?

- A. Risk of information processing
- B. Self-care deficits
- C. Disturbed thought processes
- D. Disturbed personality identity

ANSWER: B

31. A 19-year-old male has been brought to a psychiatry department for mumbling and smiling for no observable reason. He sometimes speaks like "I cannot do that; you have no power," although no one is there with him. A psychiatric nurse is planning to help the patient.

What is the most appropriate nursing intervention for this client?

- A. Determine what the client is experiencing
- B. Engage the client in a reality-based activity
- C. Teaching the client to talk back to the voices forcefully
- D. Help him to talk with other clients who have similar experiences

ANSWER: A

32. A patient was brought to a psychiatry clinic by his family with a complaint of being suspicious of his family. He refuses to take food from his family for suspecting them of adding poison to his food.

What is the most appropriate nursing intervention in this case?

- A. Inform the family to give him food forcefully to save his life
- B. Provide canned foods to the family and allow the patient to open them
- C. Give him ample time to decide on his own
- D. Tell the family to give nutritious diets

ANSWER: B

33. A 21-year-old patient on a high dose of antipsychotic was brought to the psychiatry department with a complaint of refusing to speak. In spite of the high dose she was taking, she was still symptomatic. On evaluation, a psychiatric nurse identified multiple seizure attacks that preceded the psychotic symptoms. She had a history of multiple seizures. Psychosis due to epilepsy was considered a diagnosis.

What is the most appropriate primary nursing intervention for this patient?

- A. Stop medication and notify the physician
- B. Increase the dose of antipsychotic
- C. Combine psychotherapy with antipsychotic
- D. Advise the patient to continue taking medication

ANSWER: A

34. A 20-year-old male client was brought to a psychiatry department by his parent for poor social interaction and unusual behaviour. His parent reported that he had been looking at them strangely as if he did not know them, refusing to talk to anyone, spending a lot of time in his room alone. On mental status examination, the psychiatric nurse identified a commanding type of hallucination.

What is the most appropriate nursing intervention for this patient?

- A. Show attitude of acceptance
- B. Reinforce his hallucination
- C. Use physical restraints to avoid the risk of injury
- D. Give him haloperidol 5 mg IM and diazepam 10 mg IV stat

ANSWER: A

35. A patient with a known diagnosis of schizophrenia is on an adequate dose of haloperidol. However, he still has delusional thoughts that have made him preoccupied with people who are around, as a result of which he is not willing to get along with others.

What is the most appropriate nursing intervention for this patient?

- A. Reinforce and focus on reality
- B. Discuss his delusional belief
- C. Tell the patient you view his idea as real
- D. Consider psychotherapy rather than medication

ANSWER: A

36. After two days of treatment with haloperidol 5 mg per day, a 42-year-old was presented to a psychiatry OPD with complaints of muscle rigidity, restlessness, and unable to speak. The vital signs and laboratory examinations revealed hyperthermia, increased blood pressure, and increased levels of creatinine phosphokinase.

What is the most likely side effect of the medication taken in this case?

- A. Tardive dyskinesia
- B. Neuroleptic malignant syndrome
- C. Acute dystonia
- D. Medication-induced acute akathisia

ANSWER: B

37. A 43-year-old accountant has a body mass index of 17.5 Kg/ M^2 . She has a history of active and residual symptoms of psychosis. She has been taking Haloperidol 1.5mg PO BID for the last 4 months. On evaluation, a clinician objectively finds lead pipe rigidity, perioral tremor, stooped posture, and shuffling Gait.

What is the most likely side effect identified in this case?

- A. Neuroleptic-induced Parkinsonism
- B. Neuroleptic-induced dyskinesia
- C. Neuroleptic-induced akathisia
- D. Neuroleptic-induced dystonia

ANSWER: A

38. A 40-year-old woman manifested rigidity of hands and neck, decreased level of consciousness, and rigidity of muscles after initiation of antipsychotic medication a month ago. Vital signs showed Blood pressure of 170/100 mmHg, pulse rate of 118 beats/min, and temperature of 39.5 degrees centigrade. On laboratory examination, her creatinine phosphokinase was high.

What is the most appropriate management for this patient?

- A. Trihexyphenedyl
- B. Amantadine
- C. Switch the medication
- D. Decrease the dose of antipsychotic

ANSWER: B

39. A 45-year-old male visited a psychiatry outpatient department for evaluation of medication side effects. The patient was on haloperidol 2.5mg PO BID for 2 years. He complains of abnormal body movements like facial grimacing, limb, jaw, and finger movements. He has reported that the symptoms worsen when he is highly stressed and disappear during sleep.

What is the LEAST LIKELY treatment approach for the above case?

- A. Vitamin
- B. Diazepam
- C. Benztropine
- D. Trihexyphenidyl

ANSWER: C

40. A 47-year-old married male was hospitalized for the first time with a diagnosis of schizophrenia with insidious onset. He had positive symptoms of psychosis and depression. The history revealed that he had a family history of mood disorders.

What is the most likely prognosis of the above case?

- A. The patient has a good prognosis
- B. The patient has a poor prognosis
- C. The patient has an uncertain prognosis
- D. The prognosis can be good or bad

ANSWER: A

41. A 29-year-old man was brought by his elder brother to a psychiatry clinic with complaints of hearing of voice in the absence of external stimuli, disturbance of sleep, and refusal to eat because he thought that somebody might poison the food. Those signs and symptoms persisted for 6 months. On mental status examination, the patient appears depressed and reports a loss of interest. He has no history of substance use or concurrent medical conditions.

What is the most appropriate management for this patient?

- A. Haloperidol
- B. Clonazepam
- C. Amitriptyline
- D. Clozapine

ANSWER: A

42. A 28-year-old female with schizoaffective disorder on carbamazepine recently developed excessive worry, loss of interest, depressed mood, and impaired concentration. On mental status examination, the psychiatrist has identified psychomotor agitation and worthlessness. There is no history of substance use.

What is the most appropriate management for this patient?

- A. Add diazepam
- B. Add amitriptyline
- C. Switch carbamazepine to lithium
- D. Add fluoxetine

ANSWER: B

43. A nurse student has conducted an interview at an OPD for a new patient who speaks slowly and responds only to a few questions. The patient is unhappy and blames herself excessively. She spends more hours in bed, but feels so tired. There are no significant findings on medical conditions and substance-related issues.

What is the most relevant history to be assessed to rule out depression in this case?

- A. Mood of the patient
- B. Thought form history
- C. Motor activity
- D. Cognitive behaviour

ANSWER: A

44. A male patient, escorted by his family members, came to a psychiatry clinic. The family members reported that he had not slept for 10 days, seemed so happy, but failed to go to work. He was talking more, but jumping from one idea to the other. He said, "I do have a message from God to the people around". Though he came for the first visit, the family members reported a similar incident 2 years before that was self-limited. No significant laboratory findings were found for medical illness.

What is the most likely diagnosis of the patient?

- A. Bipolar I disorder, currently manic episode
- B. Bipolar I disorder secondary to general medical conditions
- C. Bipolar II disorder, manic episode
- D. Bipolar I disorder, depressive episode

ANSWER: A

45. A lady has brought her younger sister, who attempted to drink organophosphate poison. The patient reported that for the last 3 weeks, she often feels sad, fails to enjoy everything in her surroundings, and sleeps more hours of the day. She feels tired and has stopped her schooling. The lady also remembers periods of talking more about her ability to do everything, which lasted only four days.

What is the most likely diagnosis for this patient?

- A. Bipolar II disorder, current episode depressed
- B. Bipolar I disorder, current episode manic
- C. Bipolar II disorder current episode hypo manic
- D. Bipolar I disorder, current episode mixed

ANSWER: A

46. An adult, who was taking anti-hypertensive medications for more than 10 years, has been referred to a psychiatry clinic for his behavioural changes. He gets irritated by everything at home, sleeps for only a few hours, undermines the people around him, and has been absent from the workplace for the last month.

What is the most likely diagnosis for this patient?

- A. Bipolar II disorder with depressive features secondary to hypertension
- B. Bipolar I disorder due to hypertension with manic features
- C. Bipolar I disorder with mixed features secondary to hypertension
- D. Bipolar II disorder with hypo manic features secondary to hypertension

ANSWER: B

47. A person has been brought to a hospital by his family. He wanders for no reason and talks too loudly. He speaks about his special gift from God, which no one has been given. He does not need to sleep but feels energetic. Sometimes he says that there is someone who speaks with him that others cannot hear or see. He has no sadness and related symptoms. No substance or medical history identified. The patient is diagnosed with a bipolar I disorder, manic episode with psychotic features.

What is the most appropriate differential diagnosis for this patient?

- A. Schizoaffective disorder
- B. Major depressive disorder
- C. Factitious disorder
- D. Anxiety disorder

ANSWER: A

48. A male patient on psychiatric medication has come to a psychiatry clinic with complaints of insomnia and agitation. He also has weight loss and a headache. No other medical findings are reported.

What is the most likely explanation for this finding?

- A. Side effects of SSRIs
- B. Side effects of Anxiolytic agents
- C. Side effects of carbamazepine
- D. Side effects of lithium carbonate

ANSWER: A

49. A patient, who had been on an anticonvulsant for the last 5 years, came to a psychiatry clinic with complaints of feeling sad and worthless for a month. He stopped going to school because of frustration. He also reported that he was tired easily and was sleeping more than usual. After appropriate evaluation, he was given a tricyclic antidepressant.

What is the most likely side effect the patient encountered?

- A. Seizure attack
- B. Sexual dysfunction
- C. Hypertensive crises
- D. Occipital headache

ANSWER: A

50. A patient, who is at a psychiatric OPD, complains about his sense of worthlessness and black thoughts for three months. His caretaker reported that the patient stopped going to go for work and attempted to hang himself during the night. There is no other relevant medical and substance history obtained.

What is the most likely actual nursing diagnosis for this patient?

- A. Risk for suicide related to depressed mood
- B. Risk for infection related to poor appetite
- C. Risk for contamination related to poor hygiene
- D. Risk for others related to hopelessness

ANSWER: A

51. A psychiatric patient came to a clinic for a check-up. She had an improvement in her mood and started functioning better. However, after 2 weeks, she started treatment, and she faced poor bowel habits and abdominal discomfort. She used to go to the toilet every other day. And, there was no other complaint reported. For this reason, she asked to change the medication or to stop it.

What is the most likely nursing diagnosis of this patient?

- A. Constipation related to side effects
- B. Infection related to decreased food intake
- C. Suicide is related to increased energy
- D. Injury directed at others related to commanding hallucination

ANSWER: A

52. A diabetic mother has been brought to a clinic by her daughter as she has shown behavioural changes since this morning. The daughter complains that her mother is restless, cries, and asks irrelevant questions when asked to take her medications. The mother does not remember what she said and where she is now.

What is the most likely actual nursing diagnosis of this patient?

- A. Acute confusion
- B. Impaired parenting
- C. Chronic sorrow
- D. Ineffective denial

ANSWER: A

53. A female patient has been brought to a clinic by her elder brother. She has a sad feeling and does not enjoy her life. She missed reminding me of important events. She is bored with everything in her surroundings.

What is the most likely actual nursing diagnosis of this patient?

- A. Impaired memory
- B. Risk for acute confusion
- C. Ineffective denial
- D. Risk for suicide

ANSWER: A

54. A lady has been brought to a clinic by her family with complaints of crying and sitting idle for the last 3 days. A week ago, she felt superior to others and wasted more hours on housework. In addition, she was talking too much than usual. The family reported that the first behavioural change was observed for about 4 days. There are no other medical and psychological findings identified.

What is the most likely diagnosis for this patient?

- A. Bipolar II disorder, current episode depressed, with mixed features
- B. Bipolar I disorder, current episode manic, with anxious distress
- C. Bipolar II disorder, current episode manic, with mixed features
- D. Bipolar I disorder, current episode depressed, with mild severity

ANSWER: A

55. A young patient was brought to an OPD for his unusual behaviours of 8 days duration. On mental status examination, it was identified that he had pressured speech and grandiose delusions. He was cheerful in appearance, and it was difficult to interrupt him during the interview. The family reported that he had never shown abnormal behaviour before. He had stopped going to school for a week. No other significant substance and medical history was obtained.

What is the most likely interpretation of the findings of this patient?

- A. Bipolar I disorder, most recent episode manic.
- B. Bipolar II disorder with psychotic features
- C. Bipolar II disorder, most recent episode hypomania
- D. Bipolar I disorder, most recent episode mixed

ANSWER: A

56. A 40-year-old male patient has been admitted to the psychiatric inpatient unit with complaints of suicidal attempt, decreased appetite, sleep disturbance, fatigue, guilt, and sad feelings nearly every day for the past 3 weeks. Due to such a condition, he has been failing to meet his regular duties as expected.

What is the most appropriate nursing intervention for this patient?

- A. Create a safe environment for the client
- B. Encourage verbalizations of honest feelings
- C. Encourage the client to reach out for spiritual support
- D. Maintain special care in the administration of medications

ANSWER: A

57. A nurse has witnessed a 17-year-old female patient in an inpatient department who is wearing clothes that are revealing and sometimes undressed in public areas. Upon psychiatric interview, the patient has a history of increased sexual desire supported by intrusive behaviour toward both the nurse and others around.

What is the most appropriate next step in the nursing intervention of this patient?

- A. Vigilantly redirect the client to a more socially appropriate activity
- B. Assist the client in performing aspects of self-care when required
- C. Accept the client together with her negativism
- D. Provide positive reinforcement for the client's voluntary interactions

ANSWER: A

58. A 35-year-old male patient, with a previous diagnosis of hyperthyroidism and 14th post-operative day from the surgical unit, has been linked to a psychiatric inpatient unit. He has been experiencing a low mood and fatigue nearly every day for the past 2 weeks. He also has a history of decreased appetite, sleep disturbance, psychomotor retardation and guilt feeling nearly every day. Upon interview, a nurse has been in an active process of receiving information and maintains good eye contact.

What is the most appropriate therapeutic communication technique employed by the nurse?

- A. Focusing
- B. Theme identification
- C. Silence
- D. Listening

ANSWER: D

59. A 45-year-old male patient, with a known hypertensive history for the past 1 year, has been admitted to a psychiatric inpatient unit. He has a history of low mood, psychomotor retardation and inability to make decision nearly every day for the past 1 month. He has repeatedly verbalized death wishes and wants a companion.

What is the most likely nursing intervention for this patient?

- A. Try to redirect violent behaviour by means of physical outlets
- B. Help the client recognize when anger occurs and enhance acceptance
- C. Remove all dangerous objects from the client's environment
- D. Act as a role model for the appropriate expression of angry feelings

ANSWER: C

60. A 28-year-old male patient visited a psychiatric outpatient department. He was diagnosed with major depressive disorder and is getting amitriptyline 75 mg PO noct. Upon his second visit, he has a complaint of sedation.

What is the most appropriate first management for the patient's complaints?

- A. Give the dose at bedtime
- B. Switch the medication to SSRIs
- C. Exercise program
- D. Add the dose

ANSWER: A

61. A 20-year-old male patient, who was diagnosed with a major depressive disorder and suffered from psychotic features, visited an outpatient department. He was taking amitriptyline 25mg noct. Upon follow-up, haloperidol 2.5mg was added. Then, after a few days, he developed complaints of fever, diaphoresis, rigidity, confusion, fluctuating level of consciousness, fluctuating blood pressure, and tachycardia.

What is the most appropriate management for the patient's complaints?

- A. Increase the dose of amitriptyline
- B. Withdraw Haloperidol
- C. Increase the dose of haloperidol
- D. Give diazepam

ANSWER: B

62. A 25-year-old male patient, who was diagnosed with major depressive disorder and taking amitriptyline 75 mg/day, has come to the clinic for his follow-up appointment. Upon examination, he has dizziness, feels excessively tired, and with blurred vision.

What is the most appropriate management in this case?

- A. Reduce the dose
- B. Shift to SSRIs
- C. Add artane
- D. Recommend exercise

ANSWER: A

63. A psychiatrist is treating a 30-year-old male patient at an outpatient department. The patient has pressurized speech, psychomotor agitation, and increased goal-directed activity. He also has inflated self-esteem supported by an elated mood for the past 1 week, which led to interpersonal and occupational impairment. The psychiatrist has planned to put him on sodium valproate.

What is the most effective loading dose of sodium valproate for this patient?

- A. 20 mg/kg/day
- B. 40 mg/kg/day
- C. 5 mg/kg/day
- D. 10 mg/kg/day

ANSWER: A

64. A 16-year-old male patient visited a psychiatric outpatient department with a complaint of severe recurrent temper outbursts manifested by verbally insulting others. He reported the outbursts were grossly out of proportion in intensity to the situation. He had irritability 3 times a week for the past 1 year. In addition, he also had a history of recurrent thoughts of being infected with germs, supported by washing his hands every 2 minutes. He reported this routine was time-consuming.

What is the most likely treatment option in this scenario?

- A. Fluoxetine
- B. Amitriptyline
- C. Venlafaxine
- D. Bupropion

ANSWER: A

65. A 35-year-old female visited a psychiatry outpatient department. She has been diagnosed with persistent depressive disorder, currently in full remission. She complains of a fear of heights, which is difficult to control. She also reports she has difficulty going to the workplace as it has many stairs for the past 8 months. Psychoactive substance use and chronic medical history have been ruled out.

What is the most appropriate psychotherapy for this patient?

- A. Behavioural therapy
- B. Aversion therapy
- C. Psychoanalytic therapy
- D. Narrative therapy

ANSWER: A

66. A 34-year-old male patient visited a psychiatry outpatient department. He has been diagnosed with major depressive disorder, is currently in full remission, and has been on amitriptyline 75mg PO noct. He complains of inability to obtain an erection during sexual activity, and maintaining an erection until the completion of sexual activity for the past 6 months after treatment.

What is the most effective management in this scenario?

- A. Increase the dose of amitriptyline
- B. Change treatment to Trazodone
- C. Augment diazepam
- D. Augment antipsychotics

ANSWER: B

67. A 20-year-old female patient has been admitted to a psychiatry inpatient unit with a diagnosis of major depressive disorder, in partial remission. She is currently taking Fluoxetine 20 mg PO morning. Upon interview, she reported a pervasive and excessive need to be taken care of, which led to clinging behaviour since she was a teen. She also has a fear of separation, difficulty making everyday decisions without the help of others' advice, and needs others to assume responsibility for most major areas of her life.

What is the most effective psychotherapy for this patient?

- A. Narrative therapy
- B. Dialectical therapy
- C. Interpersonal therapy
- D. Insight-oriented therapy

ANSWER: D

68. A patient, who is on psychiatric treatment, came to a psychiatry unit for his second visit. At his first visit, he was cheerful, having a problem of too much talk and multiple ideas in his mind, pressuring him to speak them out. He has been taking the medications as ordered, and no side effects have been reported.

Which of the following most likely shows the effectiveness of the patient's therapy?

- A. Maintaining appropriate weight
- B. Maintaining stability of mood
- C. Maintaining safety of others
- D. Maintaining high water intake

ANSWER: B

69. A female patient has come to a psychiatry OPD for a check-up. Previously, she preferred to be alone in her room with crying and feeling worthless, and she spoke only a few words to the interviewer. No side effects of medications have been reported now.

What is the most likely outcome criterion to be assessed for this patient?

- A. Willing participation in activities and increased socialization
- B. Presence of skin darkness in areas exposed to direct sunlight
- C. Laboratory check-up for the medication serum level
- D. Has experienced no injury to harm others

ANSWER: A

70. A young patient, who was taking Lithium carbonate, has been brought to an emergency room. The family members have reported that he went to a rural area three months ago and missed his laboratory check-up. Now he is confused. On examination, there is sluggish motor activity and increasing tremors.

What is the most appropriate next step in the management of this patient?

- A. Immediately discontinue the medication
- B. Decrease the water intake of the patient
- C. Increase the dose of the existing medication
- D. Give the medication with milk to lower GI upsets

ANSWER: A

71. A lorry driver, who is taking lithium, has been brought by his wife with a complaint of sleepy behaviour during working hours. He was saved from a car accident twice by the help of his assistant. He started the medication a month ago. There is no identified related medical problem.

What is the most appropriate preventive measure for this condition?

- A. Encourage the patient to increase his fluid intake.
- B. Stop driving until lithium levels are stabilized.
- C. Advice to rise slowly from a sitting or lying position
- D. Avoid excessive use of beverages containing caffeine

ANSWER: B

72. A female patient has come to a psychiatry OPD. She was on a tricyclic antidepressant for the last ten days. She complains of vision disturbance even during the day. She does not report any other medication side effects.

What is the most effective management for this patient?

- A. Offer reassurance that it will be resolved in a few weeks
- B. Order high-quality eyeglasses and inform them to use them strictly
- C. Increase fluid intake if not contraindicated
- D. Monitor blood pressure more frequently and document it

ANSWER: A

73. A 13-year-old boy has been brought to a psychiatry OPD by his mother for his frequent fear of dogs in his village and on the way to school. On queries, his mother reported that her son was often back home crying and missing his books because of excessive and inappropriate fear of dogs, and he even stopped going to school for the last two months.

What is the most likely additional information needed for the diagnosis of this case?

- A. Age at first onset
- B. Total duration of illness
- C. Assessing history of illness
- D. Assessing psychosocial factors

ANSWER: B

74. A 30-year-old woman has come to a psychiatry clinic with complaints of repeated and intense surges of sudden shortness of breath, palpitation, hand shakiness, and abdominal discomfort lasting for several minutes for the last several years. There is no stimulus preceding these presentations. Her physical examination and laboratory investigation have revealed negative results. What is the most likely additional symptom needed for the diagnosis of this patient?
- A. Tingling sensation of the extremities
 - B. Worry about an additional attack
 - C. Experiencing extreme heat sensation
 - D. Fear of being negatively evaluated
- ANSWER: B
75. A 35-year-old man came to a psychiatry clinic with a complaint of frequent uncertainty about locking his door and associated repeated checking of the door. He reported that this activity was consuming his time for not least two hours per day. In history, he reported that his father had the same problem. He also stated that he used to drink alcohol for several years. As per his report, these symptoms significantly affected his daily routines at the workplace. What is the most likely contributing risk factor in this case?
- A. Being male
 - B. Family history
 - C. History of alcohol use
 - D. Being at the typical age of onset
- ANSWER: B
76. A 25-year-old government employee and divorced man has come to a psychiatry clinic with complaints of recurrent nightmares and flashbacks after surviving from horrific car accident, which resulted in the death and injury of several passengers two months back. He experienced a right leg fracture that was managed by surgical intervention. He is currently living alone. After a thorough assessment, a clinician has come up with a diagnosis of PTSD. What is the most likely risk factor contributing to the situation of the patient?
- A. Being male
 - B. Experiencing a fracture
 - C. Poor social support system
 - D. Having surgical intervention
- ANSWER: C
77. A 33-year-old woman, who was diagnosed with panic disorder, has come to a psychiatry clinic for regular follow-up with complaints of feeling tired easily, palpitations, shortness of breath, and choking-like sensations despite good adherence to her medication for several months. This has made her treating clinician suspect the underlying potential causes for her complaints. What is the most likely investigation needed for this patient?
- A. Brain imaging
 - B. Liver function test
 - C. Renal function test
 - D. Echocardiography
- ANSWER: D

78. A 19-year-old girl has come to a psychiatry clinic with a spell of pounding heart and associated fainting lasting for ten minutes and remits then after. Her elder sister reported that she remained unresponsive and had jerks in her extremities, and screamed aloud while fainting. She mentioned that she suffered a lot because of the frequent occurrence of this condition.

What is the most likely laboratory investigation needed for this patient?

- A. Electroencephalogram
- B. Thyroid function test
- C. Renal function test
- D. CT scan

ANSWER: A

79. A 27-year-old male has come to a psychiatry clinic with complaints of headache and difficulty falling asleep for the last ten months. On further assessment, he mentioned extreme worry about his job performance despite his smooth execution of routine office activities and associated difficulty paying attention to his work. He also complains of chronic muscle pain and abdominal discomfort since the onset of the current illness. Laboratory investigation and physical examination findings remain normal.

What is the most likely diagnosis for this case?

- A. Agoraphobia
- B. Panic disorder
- C. Social anxiety disorder
- D. Generalized anxiety disorder

ANSWER: D

80. A 19-year-old male 3rd year University student came to a psychiatry clinic with a complaint of fear that he frequently experiences while conducting presentations in front of his fellow students. While conducting a presentation, he was experiencing palpitations, shakiness of the body, dry mouth, and associated slurring of speech. He had a fear of negative judgment that his fellow students might pose to him when he became anxious in such a situation. These presentations lasted for a year.

What is the most likely diagnosis for this case?

- A. Generalized anxiety disorder
- B. Social anxiety disorder, performance type
- C. Specific phobia, situational type
- D. Agoraphobia

ANSWER: B

81. A 30-year-old businessman does not want to travel by airplane because of the fear of danger that a plane may face crash accident. Because of this, he often misses urgent business meetings that require travel overseas. In addition, he merely uses the stairs to his hotel room, avoiding the use of the elevator because of the fear of a sudden accident it may cause. He had been with these presentations for many years and had no history of trauma before.

What is the most likely diagnosis for this case?

- A. Specific phobia
- B. Specific phobia
- C. Agoraphobia
- D. Acrophobia

ANSWER: A

82. A 35-year-old man has come to a clinic with a complaint of an urge that comes to his mind with the content of stabbing people around. This urge causes great discomfort because he experiences it against his will. He strongly avoids knives and other sharp materials, if any, in his surroundings. He often engages in praying to alleviate distress from the urge, and he is ambivalent about the falseness of his experience. These symptoms bothered him a lot, lasting for three years.

What is the most likely diagnosis for this case?

- A. Obsessive-compulsive disorder with poor insight
- B. Obsessive-compulsive disorder with absent insight
- C. Obsessive-compulsive disorder with delusional belief
- D. Obsessive-compulsive disorder with good or fair insight

ANSWER: D

83. A 22-year-old woman has come to a clinic with a fear of standing in line and attending the cinema without a companion. She reports her apprehension about the unavailability of help when she experiences panic-like symptoms in such a situation, and she frequently refuses to face such situations.

What is the most likely actual nursing diagnosis of this case?

- A. Fear related to the absence of help during the cinema stay
- B. Fear related to standing in line, as evidenced by refusal
- C. Fear related to cinema, as evidenced by avoidance of it
- D. Fear related to the absence of help, as evidenced by refusal to attend the cinema

ANSWER: D

84. A 30-year-old man came to a clinic with a debilitating fear of traveling by airplane and attending the cinema for several years. He claimed that he wanted to avoid using a plane and attending the cinema due to his intense fear of plane crash accidents and being suffocated while in a crowd, respectively. A clinician became confused about whether to come up with a diagnosis of agoraphobia or a specific phobia.

What is the most likely symptom for the diagnosis of agoraphobia rather than a specific phobia?

- A. Fear of a single situation
- B. Fear of dying
- C. Fear of being harmed
- D. Fear of being embarrassed

ANSWER: C

85. A 23-year-old university female student experienced a car accident on her way back home between the semester break. There was no death incidence; only a few sustained major and minor physical injuries occurred to her friends. At that time, she felt intense fear and helplessness. Two weeks following the incident, she started to experience the event as it was happening by then, and avoided talking about the event so as not to remember the incident.

What is the most appropriate nursing intervention for this case?

- A. Help the patient refocus on reality
- B. Decrease stimulation during an episode of anxiety
- C. Tell the patient not to stop taking medication without the practitioner's instructions
- D. Help the patient identify triggers that cause stress related to the traumatic event

ANSWER: D

86. A 35-year-old man witnessed a pedestrian being hit by a car three months ago. He has come to a psychiatry clinic and tells a psychiatry nurse that he has difficulty sleeping because of flashbacks of the accident and intrusive memories of experiencing again the traumatic event. He is also not comfortable talking about the traumatic event and avoids going to the scene. Recently, he has been unable to perform his activities of daily living because of these symptoms.

What is the most appropriate nursing intervention for this case?

- A. Encourage him to talk about the traumatic event at his own pace.
- B. Teach him the signs of anxiety and develop coping skills to reduce anxiety
- C. Educate him to decrease stimulation during an episode of anxiety
- D. Permit him to take time and carry out repetitive behaviours

ANSWER: A

87. A 40-year-old woman, who was working as an administrative executive secretary, was brought to an emergency department by her colleagues due to the sudden experience of difficulty breathing, dizziness, palpitation, shakiness, and a feeling of terror that she was going to die of a heart attack. After a thorough medical evaluation, all results were normal. An emergency physician referred her for psychiatric evaluation. While assessing her, a psychiatry nurse found out that she had repeated attacks previously that lasted for several minutes and resolved by themselves without getting treatment. But the patient now reports that she is very concerned about having another attack.

What is the most appropriate management for this patient?

- A. Reassure her and send her home; symptoms will resolve by themselves gradually.
- B. Reassure her and initiate an antidepressant to prevent relapse of the symptoms.
- C. Admit her and follow her condition to rule out a medical condition.
- D. Refer her to a cardiologist for further evaluation

ANSWER: B

88. A 28-year-old married woman came to a psychiatry clinic, reporting that she felt like she was having a heart attack whenever she left her home. She began sweating and trembling and felt dizzy for about 15-30 minutes duration about five times per month. While reviewing her chart, a psychiatry nurse noted that no abnormalities were detected on the electrocardiogram (ECG) on a recent investigation.

What is the most appropriate treatment option for this case?

- A. Selective serotonin reuptake inhibitors
- B. Benzodiazepines
- C. Monoamine oxidase inhibitors
- D. Tricyclic and tetracyclic drugs

ANSWER: A

89. A 40-year-old woman has been brought to a psychiatry clinic by her husband. Her husband reported that she has been frantic and unable to leave the house during the last four weeks alone because her sister, who used to accompany her in every outdoor like the market or shopping, has been transferred to another city. She is extremely concerned about getting the stuff needed for her home because she feels unsafe in unfamiliar surroundings and overcrowding.

What is the most appropriate first-line group of drug therapy for this case?

- A. Selective serotonin reuptake inhibitors
- B. Monoamine oxidase inhibitors
- C. Tricyclic and tetracyclic drugs
- D. Benzodiazepines

ANSWER: A

90. A 30-year-old university female student experienced a car accident on her way back home during semester break. At that time, she felt intense fear and helplessness. Two weeks following the incident, she started to experience the event as it was happening then and avoided talking about the event so as not to remember the incident.

Which psychotherapy technique is the best appropriate for this case?

- A. Trauma-focused cognitive-behavioural therapy
- B. Systemic desensitization
- C. Rehearsal and exposure therapy
- D. Behavioural Therapies

ANSWER: A

91. An 18-year-old male high school student has come to a psychiatry clinic with an extreme fear of spiders for the last 10 years. He reported that every cloth touch resembles the movement of a spider on his body, and he feels distressed all the time. After assessment, he has been diagnosed with a specific phobia.

What is the most appropriate treatment of choice for this patient?

- A. Individual CBT
- B. Group therapy
- C. Systematic desensitization
- D. Supportive Psychotherapy

ANSWER: C

92. A 42-year-old man came to a clinic with dysphoric mood accompanied by feelings of anxiousness and psychomotor agitation, three months after the diagnosis of renal failure. He started dialysis four weeks ago. Recently, he stated to his parents that life was no longer worth living with such a terrible illness and a hopeless treatment option.

What is the most appropriate psychotherapy recommended for this case?

- A. Group psychotherapy
- B. Insight-oriented therapy
- C. Problem-solving psychotherapy
- D. Psychodynamic psychotherapy

ANSWER: A

93. A 55-year-old woman was presented to a psychiatry clinic accompanied by her adult son, who expressed concern about her inability to throw old things away. He reported that her home was extremely cluttered with old, broken, and needless utensils and clothes. Whenever he attempted to help her in organizing things, she would become agitated and argumentative.

What is the most appropriate treatment for this patient?

- A. Cognitive Behavioural Therapy
- B. Insight-oriented psychotherapy
- C. Individual psychotherapy
- D. Habit Reversal training

ANSWER: A

94. A 38-year-old female customer service officer at a local bank has come to a psychiatry clinic with a complaint of a long history of checking rituals. She claims that she checks her cabinet frequently and counts the money repeatedly several times even if her colleagues and the customers are at the window and tell her she is right. She often has the thought that she had not locked the cabinet and counts the money incorrectly.

What is the most appropriate first-line treatment choice for this patient?

- A. Sertraline
- B. Lorazepam
- C. Imipramine
- D. Risperidone

ANSWER: A

95. A 30-year-old female patient was referred by a primary physician for psychiatric evaluation with complaints of nervousness, emotional lability, irritability, and apprehension for several months. While reviewing her medical history, a psychiatry nurse noted that she was diagnosed with hyperthyroidism on the same day. After a detailed assessment, the psychiatry nurse concluded that the diagnosis is anxiety disorder due to hyperthyroidism.

What is the most appropriate treatment for this patient?

- A. Immediately start to treat anxiety symptoms with sertraline.
- B. Treat anxiety symptoms with psychotherapy
- C. Treat the underlying hyperthyroidism and follow her progress.
- D. Treat anxiety symptoms with pharmacotherapy and psychotherapy

ANSWER: C

96. A 30-year-old male on anxiolytic medication has come to a psychiatry clinic with the complaints of loss of sexual drive and delayed ejaculation. Six months ago, he was diagnosed with generalized anxiety disorder. Physical examination and laboratory investigation results show normal.

What is the most likely medication this patient is taking?

- A. Fluoxetine
- B. Bupropion
- C. Lorazepam
- D. Bromazepam

ANSWER: A

97. A 30-year-old male, who was diagnosed with panic disorder, came for a regular follow-up visit at the psychiatry unit of a health centre. He was on medication for the last eight weeks. During assessment, he complained of difficulty defecating and dry mouth for the last three weeks. He claimed that he experienced these problems after increasing the dose of his medication during his last visit. Physical examination and laboratory investigation results were normal.

What is the most likely medication this patient was taking?

- A. Imipramine
- B. Fluoxetine
- C. Bupropion
- D. Lorazepam

ANSWER: A

98. A patient taking an anxiolytic medication complains of excessive sweating during the night. He unusually drenches the bed sheet and changes it twice per night. He was diagnosed with an anxiety disorder two months ago and was given an anxiolytic medication. It is after the medication that he has started to experience nocturnal sweating. This bothers him a lot. Laboratory investigation shows normal.

What is the most likely medication he is taking?

- A. Fluoxetine
- B. Amitriptyline
- C. Diazepam
- D. Mirtazapine

ANSWER: A

99. A 30-year-old male came to a clinic with a complaint of difficulty falling asleep after he started taking sertraline 50mg per day. He was on sertraline for the last three weeks after being diagnosed with posttraumatic stress disorder. Physical examination and laboratory investigation results show normal.

What is the most appropriate treatment for this patient?

- A. Stop sertraline
- B. Increase the dose of Sertraline
- C. Add benzodiazepines for a short time
- D. Change Sertraline to Fluoxetine

ANSWER: C

100. A patient has come to a clinic with a complaint of loss of sexual desire associated with failure to have adequate erection for the last three months. He has been on sertraline 100mg per day for the last ten months for the treatment of PTSD. Though other symptoms improved, he is concerned about his recent experience. Laboratory investigation and physical examination results are normal.

What is the most likely appropriate management for this patient?

- A. Shifting Sertraline to Fluoxetine
- B. Increase the dose of Sertraline
- C. Treat the complaint with Sildenafil
- D. Encourage the patient to take stimulants

ANSWER: C

101. A teenager from a street with a history of chronic Opioid use was brought to a psychiatry OPD by a local NGO. After a comprehensive evaluation, he was found to be Opioid dependent and transferred to a substance rehabilitation centre.

Which Opioid receptors most likely mediated the patients' condition?

- A. μ -opioid receptors
- B. κ -opioid receptors
- C. Δ -opioid receptors
- D. α -opioid receptors

ANSWER: A

102. A young drunk man was brought to a psychiatry OPD for being verbally and physically aggressive for 2 2-hour duration. On mental status examination, it was identified that he had mood lability, impaired judgment, slurred speech, incoordination, and unsteady gait. This client had been drinking beer with his friends before developing these complaints.

What is the most likely interpretation of the mental status examination findings of this patient?

- A. Alcohol intoxication
- B. Alcohol withdrawal
- C. Alcohol induced depressive disorder
- D. Alcohol induced psychosis

ANSWER: A

103. A 28-year-old man consulted a mental health professional to advise him regarding how to refrain from drinking alcohol. He said, "I usually work at night shift as a security guard with my colleague who usually comes with a bottle of Arekie". His colleague usually urges him to have some to stand the night cold. Though tempting, he always refuses his invitation. He has never used alcohol except for drinking occasionally on holidays. His younger brother and father are alcoholics.

What is the major risk factor that needs to be considered in the advice?

- A. The age range of the client
- B. Family alcohol use history
- C. His colleague influences
- D. Accessibility of local grocery stores

ANSWER: B

104. A 27-year-old man, who recently used alcohol, presented to a clinic with symptoms of decreased level of coordinated movement, poor social judgment, mood lability, and cognitive deterioration. On a comprehensive examination, blood alcohol level was measured.

What is the most likely blood alcohol concentration level of this client in mg/dl?

- A. 20-29
- B. 30-79
- C. 80-200
- D. 201-300

ANSWER: C

105. A passer-by brought a teenager from the street to a psychiatric emergency unit for clinical evaluation. On a comprehensive assessment, a clinician found out that the patient has used an unspecified drug in large doses and has a euphoric mood followed by apathy, disregards social norms. On mental status examination, the client has slurred speech, inattention and poor memory, papillary dilation, and drowsiness. Apart from that, he has no psychiatric or medical history.

What is the most likely substance used by the teenager?

- A. Cannabis
- B. Alcohol
- C. Hypnotic
- D. Opioid

ANSWER: D

106. A hyper vigilant security guard was brought to a psychiatry OPD by his colleague after using a large amount of psychoactive substances. On arrival, he had stereotyped behaviour, euphoria with anger, and appeared less sociable. During mental status examination, he had auditory hallucinations, psychomotor agitation, tachycardia, dilated pupils, and high blood pressure.

What is the most likely substance used in this case?

- A. Cannabis
- B. Sedatives
- C. Stimulant
- D. Hallucinogens

ANSWER: C

107. A 58-year-old man came to a psychiatry OPD for consultation. He used to take substances daily for the last 4 weeks and stopped 3 days back. After ceasing the substance, he noticed increased appetite, irritability, and insomnia. He had also decreased blood pressure. There were no prominent symptoms.

What is the most likely substance the patient was using?

- A. Nicotine
- B. Opioid
- C. Cannabis
- D. Hypnotic

ANSWER: A

108. An unemployed 20-year-old man, who lived with his parents, was brought to a hospital by some friends in a state of anxiety and agitation to the psychiatry emergency department. He used a substance for the last 2 years, but this night he had consumed an excessive amount in a few minutes, and after about an hour, he appeared tense and anxious and complained that everything was changing and distorted. He has made multiple unsuccessful attempts to cut the substance, and there were times when he endangered his life to get the substance.

Which substance use disorder was the patient most likely to experience?

- A. Stimulant
- B. Alcohol
- C. Cannabis
- D. Barbiturates

ANSWER: C

109. A 28-year-old man from the street was brought to a psychiatry OPD by a local NGO for psychiatric evaluation. The man was working in a bank 5 years back and was fired from his job related to his substance use habit. He reported that starting the drug was easy, but ceasing it was almost impossible. If he did not get the drug, he feels restless and will have an achy feeling over the back, and will experience flu-like symptoms. On physical examination, he has multiple injection sites. Which substance use disorder did this patient most likely experience?

- A. Tobacco
- B. Opioid
- C. Cannabis
- D. Alcohol

ANSWER: B

110. A 34-year-old man was brought to a psychiatry emergency after an episode of alcohol withdrawal seizure and was admitted to a psychiatry ward. During the intake interview, the patient denied that his alcohol use was not a problem.

What is the most appropriate nursing intervention required for the patient?

- A. Use argument as a strategy to convince the patient
- B. Encourage the patient's effort to externalise the problem
- C. Do not allow the client to rationalise difficulties with others
- D. Encourage the client to verbalise feelings, fears, and anxieties

ANSWER: C

111. A 26-year-old adult was presented by a friend to a psychiatry emergency unit with signs of substance disorder. On a comprehensive evaluation, a mental health professional concluded a diagnosis of Cannabis intoxication delirium.

What is the most likely initial management in this case?

- A. Consult the patient on the medical side
- B. Admit the patient to the psychiatry ward
- C. Encourage insight with a motivational interview
- D. Strengthen the patient's support system

ANSWER: A

112. A 26-year-old young man with poly-substance use has recently joined a psychiatry rehabilitation centre. A clinical psychologist with caring psychiatry professional has made the first two-month care plan.

What is the most appropriate counselling approach during these first two months?

- A. Treatment sessions should focus on be cause of addiction
- B. Preferably using individual psychotherapy rather than group therapy
- C. Focusing on the day-to-day life issue of the patient
- D. Considering psychodynamic psychotherapy

ANSWER: C

113. A 57-year-old man was brought by his family to a tertiary hospital for neuro-psychiatric evaluation. A mental health professional, together with a neurologist, made a joint evaluation and diagnosed the patient as having alcoholic pellagra encephalopathy.

What is the most appropriate choice of medication to treat the patient?

- A. 50mg PO Niacin
- B. 100mg PO thiamine
- C. 10mg PO Donepezil
- D. 8mg PO Galantamine

ANSWER: A

114. A 38-year-old man was brought by his colleagues to a psychiatry emergency OPD after a single grandmal seizure. On psychiatric evaluation, the patient was found to have a long chronic alcohol use history with recent discontinuation. Apart from ceasing the alcohol for 2 days, a clinician could not identify any other underlying cause for the seizure.

What is the most likely choice of drug to control a pending seizure in this case?

- A. Phenobarbital
- B. Sodium Valprorate
- C. Chlordiazepoxide
- D. Carbamazepine

ANSWER: C

115. A 28-year-old patient has been admitted to a psychiatry ward and is taking Methadone. A psychiatric nurse wants to provide psycho education about the side effects of the drug. What is the most common side effect of Methadone?
- A. Sweating
 - B. Diarrhoea
 - C. Vomiting
 - D. Dizziness
- ANSWER: A
116. A fifth-year female medical student has come to a psychiatry OPD for consultation. She started using marijuana a year ago at the invitation of one of her best classmates. Often her friends describe her as a sensitive and impulsive person in situations where she experiences disappointment. She also assured that she started first time out of novelty, and currently she has decided to stop. What is the most likely factor that helps the patient recover fast?
- A. Peer pressure
 - B. Being female sex
 - C. Impulsive personality
 - D. Being a student
- ANSWER: B
117. A 25-year-old male patient has been brought to an emergency psychiatry clinic with complaints of crying, loss of interest in doing, social isolation, poor appetite and suicidal attempt. He also stays in a certain place for a long period of time, and he has not been talking to his family for the last 2 weeks. He has no substance use history. What is the most relevant history to diagnose this patient as the catatonia associated with depression?
- A. Insomnia
 - B. Echolalia
 - C. Weight loss
 - D. Suicidal ideation
- ANSWER: B
118. A known schizophrenic patient, who has been taking haloperidol for the last year, was brought to a psychiatry emergency unit at midnight. Upon arrival, he had severe muscular rigidity and was confused and mute. He had also fever and increased pulse, and blood pressure. Laboratory findings showed an increased white blood cell count and increased levels of creatinine phosphokinase. What is the most appropriate diagnosis in this case?
- A. Neuroleptic-induced parkinsonism
 - B. Neuroleptic malignant syndrome
 - C. Neuroleptic-induced akathisia
 - D. Neuroleptic-induced acute dystonia
- ANSWER: B

119. A 30-year-old female came to an emergency psychiatry OPD with verbal and physical aggression towards her family. She also had a history of being easily irritable, impulsivity, and having multiple suicidal attempts. As her attendants reported, she was a chronic alcohol user and used it as an eye opener, and sometimes she chews khat and smokes cigarettes when intoxicated too much.

What is the most likely risk factor for suicide based on the above scenario?

- A. Being female
- B. The patient's age category
- C. Her aggressive behaviour
- D. Substance use history

ANSWER: D

120. A 36-year-old female client came to an emergency department with abnormal body movement, repeating words when others talk to her, and she is extremely agitated. Her family stated that she was verbally and physically aggressive towards them. In the past, she had a history of hepatic encephalitis. The present investigation revealed that she has a foliate deficiency and unstable vital signs.

What is the most likely diagnosis based on the above case?

- A. Schizophreniform disorder with catatonia
- B. Catatonia associated with another mental disorder
- C. Catatonia due to another medical condition
- D. Catatonia not otherwise specified

ANSWER: C

121. A 20-year-old male patient with schizophrenia who is on clozapine came to an emergency department with complaints of dry mouth, dizziness, and weight gain. He also stated that he had been treated for an infectious disease twice within a month. A laboratory finding indicated a significant reduction in WBC.

What does the reduction of WBC indicate in this case?

- A. Common cold
- B. Agranulocytosis
- C. Acute febrile illness
- D. Drug non-response

ANSWER: B

122. A 54-year-old female merchant with psychotic disorder came to an emergency department with fever and confusion. She also stated that she had difficulty swallowing food and stiffness in her neck. She had been taking two types of antipsychotic agents over the past four weeks. Her blood pressure was unstable, and her body temperature was greater than 38 degrees centigrade.

What is the most likely risk factor for the above case?

- A. Older age
- B. Being female
- C. Low-potency antipsychotic drug
- D. Antipsychotic polypharmacy

ANSWER: D

123. A 34-year-old male college teacher with a history of severe alcohol use disorder attended an emergency OPD. He increased the amount of alcohol consumption over the years. He reported that he has consumed at least four cups of "Areke" a day for the last eight years. He suddenly stopped taking alcohol for three days and came up with symptoms such as poor appetite, hand tremor, tactile and visual hallucinations. Vital signs were deranged.

What is the most appropriate nursing care for the patient?

- A. Promote participation in the outpatient support system
- B. Assist the client in identifying alternative sources of satisfaction
- C. Provide education and assistance in the course of treatment
- D. Administer substitution therapy as ordered

ANSWER: D

124. A 35-year-old man with a history of schizophrenia, multiple episodes, came to the psychiatry emergency OPD. He had multiple admissions. When he arrived at the emergency centre, he was shouting loudly by saying "I want to kill you, do not touché me". He took medications appropriately, but no improvement. Mental status examination revealed no insight and judgment. His vital signs were stable.

What is the most appropriate management for this patient?

- A. Clozapine
- B. Haloperidol
- C. Aripiprzole
- D. Ziprasidone

ANSWER: A

125. A client with chronic schizophrenia has been brought to an emergency department by his family due to breathing and swallowing problems. This individual has been taking antipsychotics for the past seven years. His family reported that they had never seen such strange things as abnormal and irregular movements of the muscles of the head, limbs, and trunk before. Mental status examination also revealed that there were finger movement, hand clenching, twisting and protruding movements of the tongue and facial grimacing.

What is the most likely side effect noticed in this case?

- A. Neuroleptic malignant syndrome
- B. Neuroleptic-induced Parkinsonism
- C. Neuroleptic-induced Tardive dystonia
- D. Neuroleptic induced Tardive dyskinesia

ANSWER: D

126. A client with a history of schizophrenia on antipsychotic attended an emergency unit with complaints of fatigue, rigidity and dysphagia. He was taking an antipsychotic for over two months. His vital signs revealed that fluctuating blood pressure and body temperature were greater than 38 degrees centigrade, associated with profuse diaphoresis.

What is the most appropriate management for the above case?

- A. Decrease the dose of antipsychotic
- B. Withdraw antipsychotic medication
- C. Stop the previous drug and start another one
- D. Tapper the previous drug and start another one

ANSWER: B

127. A 60-year-old man with dementia of Alzheimer type came to a psychiatry clinic with a complaint of a hearing derogatory voice that was insulting him. During examination, he was disinherited, had a persecutory delusion and was violent.

What is the most likely diagnosis for this case?

- A. Schizophrenia secondary to dementia
- B. Delusional disorder secondary to dementia
- C. Major depressive disorder
- D. Delirium co-morbid with dementia

ANSWER: A

128. A 70-year-old woman was brought to an emergency department by her family because she was wandering her neighbourhood in a confused and disoriented state. Her husband and their children reported a noticeable change in the patient's mental status. On examination, she was disoriented to time and place, agitated, and confused. Her husband also reported that she was suffering from dizziness and light-headedness for many years while standing. She had paranoid delusions and was hostile to family members and caretakers. She was irritable and had difficulty sleeping at night. She failed to return to a normal state of functioning, particularly as a result of diminished performance due to forgetfulness. She has dementia as a diagnosis.

What is the most likely symptom that makes this patient hostile to family members and caregivers?

- A. Paranoid delusion
- B. Disorientation to time and place
- C. Wandering around
- D. Agitation and confusion

ANSWER: A

129. A 68-year-old female who is living alone has come to a psychiatry OPD with a complaint of suicidal thoughts. She has a history of attempting suicide several times, including a drug overdose. She faced financial difficulty and had been living alone for five years since the death of her husband.

What is the LEAST LIKELY risk factor for the above case?

- A. Advanced age
- B. Being female
- C. Living alone
- D. Early onset of the illness

ANSWER: D

130. A 75-year-old man, who was diagnosed with dementia, has come to a psychiatry clinic with low mood and persistent sadness. He also reported suicidal wishes, feeling easily tired, hopelessness and progressive decline in cognitive functioning. He has been drinking beer occasionally to socialise with his friends for many years.

What is the most likely risk factor for co-existing depressive disorder?

- A. Having hypertension
- B. Decline in cognitive functioning
- C. Age of onset of the illness
- D. History of using alcohol

ANSWER: A

131. A 30-year-old woman came to a psychiatry unit with complaints of tearfulness and suicidal thoughts of one week duration after giving birth. She also reported sleep difficulty, occasional restlessness. Her family reported that she has tried to kill her child, stating that she is hearing a voice telling her to do so. She was diagnosed with postpartum psychosis.

What is the most likely differential diagnosis for the above case?

- A. Personality disorder
- B. Mood disorder
- C. Anxiety disorder
- D. Sleep disorder

ANSWER: B

132. A 30-year-old man came to a psychiatry unit with a complaint of making mistakes most of the time, though he wished for perfection in doing his everyday activities. He told a psychiatric nurse that he felt discomfort if he was not keeping his household materials in an orderly manner. The patient reported that he was on follow-up for hypertension treatment. He has no history of substance use and symptoms of other mental illnesses. After a thorough assessment, the psychiatry nurse diagnosed him with obsessive-compulsive personality disorder, comorbid with hypertension.

What is the most effective psychotherapy for this patient?

- A. Interpersonal psychotherapy
- B. Cognitive behavioural therapy
- C. Insight-oriented psychotherapy
- D. Group psychotherapy

ANSWER: B

133. A 23-year-old male university student was presented to a psychiatry clinic with complaints of decreased interest in eating and occasional suicidal thoughts. He also reported that he was unable to remember what he had learnt in the classroom, and difficult to sit, to read with concentration for longer hours and could not remember recent and past events. He claimed that he had difficulty remembering the names of familiar persons and places, but not the time. After a thorough examination, a psychiatrist realised that the patient's symptoms were simulated for dementia. Finally, he diagnosed the patient with Factitious Disorder, putting dementia on the differential diagnosis.

What is the most likely supporting symptom of Factitious Disorder?

- A. Memory loss for place
- B. Loss of remote memory
- C. Recent and remote memory loss
- D. Erratic and Inconsistent manner of symptoms

ANSWER: D

134. A 30-year-old male, who was on a follow-up visit for schizophrenia, has become disoriented to time and person. This symptom is characterised by getting improved during the daytime and worsening at night. He reported that he had difficulty falling asleep. The patient reported to a psychiatrist that he took a higher dose of the medication given as a prophylaxis for the prevention of side effects. The psychiatrist confirmed that it was the anticholinergic drug (trihexyphenidyl). He was diagnosed with medication-induced delirium beside schizophrenia.

What is the most appropriate treatment for this patient?

- A. Physostigmine salicylate
- B. Haloperidol
- C. Diphenhydramine
- D. Dantrolene

ANSWER: A

135. A 30-year-old woman came to a psychiatry OPD with complaints of psychotic symptoms during her postpartum care visit. A psychiatrist ordered an antipsychotic drug for her. A few months after taking the drug, she reported that her weight increased by 15percent and her Body mass index was 30 kg /m².

What most appropriate laboratory investigation required for the patient in this case?

- A. Lipid profile
- B. Liver function test
- C. Thyroid function test
- D. Electrolyte test

ANSWER: A

136. A 25-year-old female who was in the second week of delivery came to a psychiatry clinic with complaints of difficulty falling and maintaining sleep, followed by difficulty concentrating at daytime.

What is the most appropriate nursing diagnosis for this case?

- A. Risk of violence to her child related to hallucination
- B. Thought processes impairment related to biological changes
- C. Disturbed sleep pattern related to insomnia
- D. Social isolation related to shame is associated with symptoms

ANSWER: C

137. A 68-year-old female who is living alone presents to a psychiatry OPD with a complaint of suicidal thoughts. She has a history of attempting suicide several times, including a drug overdose. She faced financial difficulty and had been living alone for five years since the death of her husband.

What is the LEAST LIKELY risk factor for the above case?

- A. Advanced age
- B. Being female
- C. Living alone
- D. Early onset of the illness

ANSWER: D

138. A 65-year-old retired accountant came to a psychiatry OPD and got a diagnosis of late-onset schizophrenia. On mental state examination, he reported that his neighbours did witchcraft on him, and he also saw them while they were doing witchcraft to others, but nobody witnessed the event.

What is the most likely explanation for the information obtained on mental state examination?

- A. Auditory hallucination
- B. Visual hallucination
- C. Persecutory delusion
- D. Tactile hallucination

ANSWER: B

139. A 70-year-old retired man with good social support was admitted to a psychiatry ward with major depressive disorder. His complaints were decreased need for work, low mood, self-isolation, sleep difficulty, lack of energy and suicidal ideation. His spouse had died one year before the onset of these symptoms. He denied using substances and had no previous history of mental illness.

What is the appropriate nursing intervention for this case?

- A. Assess the patient's support network
- B. Assess the significant losses the patient has experienced
- C. Encourage verbal expression of feelings

D. Remain inconsistent in approach and expectations

ANSWER: C

140. A 35-year-old woman, who had a history of bipolar I disorder manic episode, came to a psychiatry OPD with a relapse of the disorder. On mental state examination, she is cheerful, talkative, agitated and jumps from topic to topic while talking. A psychiatry nurse has ordered a urine HCG test, and she is positive for HCG. She has no history of substance use.

What is the most appropriate drug to be given in this case?

- A. Valproate
- B. Olanzapine
- C. Carbamazepine
- D. Lithium

ANSWER: B

141. A 30-year-old woman, who gave birth a week ago, has come to a psychiatry unit with the complaints of unhappiness, difficulty falling asleep and severe suicidal thoughts. A psychiatrist has diagnosed her with major depressive disorder. She told the psychiatrist that she had a similar past episode for which she took different medications, but could not get adequate improvement.

What is the most effective treatment for her current episode?

- A. Tricyclic antidepressants
- B. Selective serotonin reuptake inhibitors
- C. Electroconvulsive therapy
- D. Second-generation antipsychotics

ANSWER: C

142. A 30-year-old mother has been diagnosed with postpartum depression. She is taking one of the antidepressants that cause tachypnea, cyanosis, irritability, and poor sucking reflex on her new born.

What is the most likely drug she is taking?

- A. Amitriptyline
- B. Fluoxetine
- C. Venlafaxine
- D. Phenelzine

ANSWER: A

143. A 35-year-old female patient visited a psychiatry outpatient department. In the interview, she has had pervasive distrust and suspicion of others since adolescence. She also bears grudges and suspects that others are exploiting her without sufficient basis. She is also preoccupied with unjustified doubts about the loyalty of her friends, so she is reluctant to confide in others.

What is the most likely finding on the thought content of this patient?

- A. Idea of reference
- B. Suicidal ideation
- C. Obsessional thoughts
- D. Peculiar thinking

ANSWER: A

144. A 34-year-old male patient was brought to a psychiatry outpatient department by his wife. She stated that he is jealous of her without sufficient basis, and he has been very suspicious of everyone around him for the last several years. After evaluation, a psychiatry nurse diagnosed him with paranoid personality disorder.

What is the most likely risk factor in this case?

- A. Age
- B. Being male
- C. Being married
- D. Conflict with his wife

ANSWER: B

145. A 26-year-old male patient visited a psychiatry outpatient department. During the interview, he could not relax and was scanning the examination room. He was humourless and serious. He also had a history of ideas of reference. He had restricted affect and was baffled about having to seek psychiatric help.

What is the most likely personality disorder of this patient?

- A. Paranoid personality disorder
- B. Schizoid personality disorder
- C. Schizotypal personality disorder
- D. Antisocial personality disorder

ANSWER: A

146. A 55-year-old male patient has been referred to a psychiatry unit for evaluation. He is accompanied by his wife, and he works as a school principal. His wife complains that he has a pervasive pattern of grandiosity and a need for excessive admiration. At the waiting room, he claims to get the most special doctor to see him, and he does not want to wait; he claims to get priority since he considers himself a very important person. He has been serious and lacks empathy during the interview. He also thinks others are envious of him. He also believes that he is special and among a prestigious group.

What is the most likely diagnosis for this patient?

- A. Histrionic personality disorder
- B. Dependent personality disorder
- C. Narcissistic personality disorder
- D. Avoidant personality disorder

ANSWER: C

147. A 28-year-old female patient with a diagnosis of avoidance personality disorder visited a psychiatry outpatient unit. In the interview, she reports she is unwilling to get involved with people unless certain of being liked. She reports that social situations almost always provoke anxiety. She becomes nervous and tense until she is reassured by a practitioner.

What is the most appropriate differential diagnosis for this patient?

- A. Social anxiety disorder
- B. Specific phobia
- C. Agoraphobia
- D. Dependent personality disorder

ANSWER: A

148. A 55-year-old male patient with a diagnosis of narcissistic personality disorder has come to a psychiatry outpatient department. Upon interview, he has grandiosity and a need for admiration. He also thinks others are envious of him and exaggerates his achievements. He also believes that he is superior without commensurate achievements.

What is the most differential diagnosis of this patient?

- A. Bipolar I disorder
- B. Schizophrenia
- C. Cyclothymic disorder
- D. Schizoaffective disorder

ANSWER: A

149. A 25-year-old patient came to for psychiatry unit for psychiatric evaluation with complaints of unusual perceptual experiences and a lack of close friends. The family reported that he has used alcohol and khat for 15 years old. During examination, he has constricted affect, suspicious and has been diagnosed with schizotypal personality disorder co co-morbid depression.

What is the most appropriate management for this case?

- A. Group Psychotherapy and Anti-depressant
- B. Anti-depressant and Benzodiazepam
- C. Benzodiazepam and psychotherapy
- D. Anti-depressant and interpersonal psychotherapy

ANSWER: D

150. A 40-year-old man visited a psychiatry OPD with a complaint of inability to have a good relationship with his colleagues and neighbours. He stated that they would attack him at his workplace. He was usually absent from his work to reduce the risk of being harmed by these people. A psychiatric nurse diagnosed with paranoid personality disorder and recommended to attend the follow-up treatment for his condition.

What is the most likely prognosis of this patient?

- A. He tends to have lifelong problems /a harbinger of schizophrenia
- B. Toleration of intimacy is an acceptable condition for patients
- C. The patient's treatment requires starting with group psychotherapy
- D. Interpretation of dependence and sexual concerns increase patients' trust

ANSWER: A

151. A Psychiatry nurse evaluated a 23-year-old young man who presented to the psychiatry OPD with being disappointed by his sleep pattern associated with difficulty maintaining sleep four times per week, which eventually disturbed his daily life. On examination, he has poor concentration and attention.

What is the most likely risk factor in this case?

- A. Early onset of the illness
- B. Worry-prone personality
- C. Being a male
- D. Antisocial personality

ANSWER: B

152. A 32-year-old patient, who came to a psychiatry clinic to be assessed in pursuit of a solution for delayed ejaculation disorder, reported that he had achieved ejaculation during coitus with great difficulty, which was painful for his wife for the last seven months.

What is the most likely risk factor in this case?

- A. Stressful life event
- B. Poor interpersonal relationships
- C. Advanced age
- D. Disagreement with life

ANSWER: C

153. A 30-year-old male university student visited a psychiatry clinic for a possible paraphilia disorder. He reported intense, frequent, strange sexual fantasies, ashamed to explain. He also reported that his whole life was affected by this situation.

What additional information is needed to diagnose paraphilia disorder in this case?

- A. Distress or interpersonal difficulty
- B. Somatic symptom
- C. Lack of interest in talking about the issue
- D. Lack seeking help for the feelings

ANSWER: A

154. A 20-year-old female university freshman student was brought to a psychiatry clinic by her family with extreme weakness because of fasting and excessive exercise, fearing that she would be overweight. She refuses to eat and dumps the food in the garbage. This experience lasted for one year, and her parents are concerned about her body weight and appearance. A psychiatric nurse assessed for anorexia nervosa.

What is the most likely condition that resulted in this situation?

- A. Intense fear of gaining weight
- B. Recurrent and frequent binge eating with vomiting
- C. Once a week, binge eating
- D. Binge eating without vomiting

ANSWER: A

155. A 45-year-old patient came to the psychiatry unit with a complaint of erectile problem. He was a father of three and had been performing well, but recently he could not attain his previous long-lasting sexual status needed to satisfy his wife. Because of this, his wife complained about his changing behaviour and was in frequent arguments. During examination, there were no pertinent findings, and he did not have a history of substance abuse.

What is the most likely diagnosis of the above case?

- A. Sexual dysfunction, situational subtype
- B. Sexual dysfunction, generalised subtype
- C. Sexual dysfunction, acquired subtype
- D. Sexual dysfunction, lifelong subtype

ANSWER: C

156. A 38-year-old single woman presented to a psychiatry clinic with a complaint of inability to have intercourse because of spasm and tightening of the pelvic floor muscles during attempted vaginal penetration. Even though she experienced many times, she described episodes with a recent boyfriend in which he had tried vaginal penetration. The boyfriend did not have erectile dysfunction. The woman showed sexual desire and was able to achieve orgasm through manual stimulation. What is the most likely diagnosis of the above case?
- A. Hypoactive sexual desire disorder
 - B. Delayed orgasmic disorder
 - C. Penetration disorder
 - D. Sexual aversion disorder
- ANSWER: C
157. A 58-year-old married man was referred to a psychiatry unit having a strong dislike of one's sexual anatomy. The problem is becoming worse, and he has a strong desire to be labelled as female. Most of the time, he has a strong preference for cross-gender roles and wearing as well as preferring to play opposite gender games. What is the most likely diagnosis of the above case?
- A. Transvestic disorder
 - B. Gender dysphoria
 - C. Transvestic disorder
 - D. Somatic symptom disorder
- ANSWER: B
158. A 40-year-old driver came to a psychiatry clinic with a complaint of a sudden attack of excessive and irrepressible need to sleep at inappropriate places. The episode occurred in different situations and at inappropriate times, like during eating, talking, and driving, followed by a sudden bilateral loss of muscle tone for a brief moment while consciousness was maintained. Because of this, he was suspended from driving his company's automobile. This problem persisted for the last eight months, and the person did not have a history of substance and medical problems. What is the most likely diagnosis of the above case?
- A. Narcolepsy
 - B. Hypersomnolence disorder
 - C. Sleep deprivation
 - D. Kleine-Levin Syndrome
- ANSWER: A
159. A teenage girl who was repeatedly sexually abused by her alcoholic stepfather was presented to a psychiatry OPD with a complaint of a suicidal attempt. Following the last sexual abuse, she suddenly ran away from home. Intending to go to her grandmother's home, she travelled by public transportation and walked the streets, apparently far away without attracting attention. After approximately ten hours, she was stopped by the policeman during a curfew check, and she could not recall recent events and gave her current address by insisting that she was living with her grandmother. What is the most likely diagnosis for this case?
- A. Dissociative amnestic disorder
 - B. Dissociative fugue
 - C. Dissociative identity disorder
 - D. Malingered dissociative identity disorder
- ANSWER: B

160. A 40-year-old married man, who was driving a city bus, came to a psychiatry unit with a complaint of intense memory disruption of identity. He noticed recurrent gaps in the recall of everyday events and important personal information. He said that he suddenly became a depersonalised observer of his speech and actions, which he felt powerless to stop. The patient described a family history of brutal beatings and physical disciplining.

What is the most likely diagnosis of the above case?

- A. Dissociative amnestic disorder
- B. Dissociative Identity Disorder
- C. Dissociative fugue
- D. Malingered Dissociative Identity Disorder

ANSWER: B

161. A regional psychiatry department has organised a meeting with relevant stakeholders to plan future activities for the next ten years. The intention is to determine what the department wants to achieve and how it will get there. The department also tries to find the best future and the path to reach that destination.

What is the most likely type of plan undertaken in this case?

- A. Futuristic
- B. Strategic
- C. Tactical
- D. Midterm

ANSWER: B

162. A psychiatry unit in a primary hospital has the intention to prepare a long-term plan to reduce substance abuse in a nearby kebele. During the assessment, the kebele has identified that there is a strong peasants' association established for the kebele administration's purpose.

What is the most likely strategic tool identified in this case?

- A. Threat
- B. Weakness
- C. Opportunity
- D. Strength

ANSWER: C

163. A psychiatry department head encourages involvement of team members and wisely delegates decision-making authority. He never loses sight of the fact and tries to bear the crucial responsibility of leadership. He also motivates team members by empowering and guiding them with a loose rein. What is the most likely leadership style the head followed?

- A. Laissez-faire
- B. Autocratic
- C. Anarchic
- D. Participative

ANSWER: D

164. A newly appointed psychiatry unit head can visualise the organisation as a whole, discern interrelationships among organisational parts, and understand how the organisation fits into the wider context of the unit, community, and the world. He also sees the organisation as a whole and understands the relationships among various subunits.

What is the most likely type of managerial skill described in this scenario?

- A. Technical
- B. Human relations
- C. Conceptual
- D. Communication

ANSWER: C

165. A hospital provides preventive and curative services, emergency surgical services, and serves as a practical training centre for nurses and other paramedical health professionals.

What is the most likely level of care the hospital assumes in the current Ethiopian health care delivery system?

- A. Primary
- B. Tertiary
- C. Secondary
- D. Quaternary

ANSWER: A

166. A newly assigned head of a psychiatry department is trying to bring remarkable change in the working environment. He is energetic, communicates his plans through presentations, interviews, employee evaluations, and holds press conferences.

What is the most likely communication skill demonstrated in this case?

- A. Written communication skills
- B. Oral communication skills
- C. Feedback skill
- D. Listening skills

ANSWER: B

167. A recently appointed manager in a psychiatry unit of a hospital attends ceremonies, signs documents, controls staff and creates a chain of communications in and outside the organisation.

What is the most likely role the manager played in this case?

- A. Interpersonal
- B. Informational
- C. Entrepreneur
- D. Decision making

ANSWER: A

168. A psychiatric health team was established to assess the quality of psychiatric services in a hospital. The team developed a high degree of solidarity and positive feelings towards their team. Less tension, hostility and strong belongingness were also observed in the team.

What is the most likely characteristic of an effective team observed in the case?

- A. Shared decision-making
- B. Cohesiveness
- C. Communication
- D. Flexibility

ANSWER: B

169. A primary hospital manager established a team responsible for dealing with record-keeping problems in different health care delivery settings. After a week of team establishment, he held a discussion and was able to identify cohesion, member contribution acknowledgement, and willingness to change their preconceived ideas or opinions on the basis of the facts presented by other members.

What is the most likely stage of group formation in this case?

- A. Storming
- B. Norming
- C. Performing
- D. Adjourning

ANSWER: B

170. A regional health office has planned to conduct a performance evaluation of psychiatry units in the hospitals within the region. The evaluation includes measuring intermediate-level changes among beneficiaries of the units. It also gives emphasis to building effective relationships.

What is the most likely type of evaluation conducted?

- A. Outcome
- B. Impact
- C. Process
- D. Input

ANSWER: A

171. A psychiatry nurse student learnt that eating disorder was a significant public health problem among school-age children. The student reviewed relevant literature and found adequate evidence overseas, but not enough information available in his country. Finally, the student decided to conduct research on eating disorders among school-age children in a district.

What is the most likely title selection criterion in this case?

- A. Applicability
- B. Relevance
- C. Novelty
- D. Feasibility

ANSWER: C

172. Researchers identified the adult population with depression to provide cognitive behavioural therapy. Then, they randomly classified study participants into two groups: the intervention group, which received cognitive behavioural therapy and the control group, which received the usual care. The researchers followed both groups for 12 months to measure the outcome.

What is the most appropriate study objective in this case?

- A. To evaluate the effect of cognitive behavioural therapy
- B. To identify determinants of depression among patients
- C. To compare the health outcomes of depression
- D. To determine the prevalence of depression

ANSWER: A

173. A student observed frequently patients with common mental disorders during his clinical attachment at a hospital. Then, he found little evidence of the disease in the literature review and determined to conduct the burden and its associated factors of this health problem among patients who visited the hospital within a month.

What is the most appropriate study design in this case?

- A. Cross-sectional
- B. Case control
- C. Cohort
- D. Ecological

ANSWER: A

174. An investigator determined the prevalence of substance use disorder using 1200 adults drawn from a total population of 6000 females and 3500 males of a district. The investigator included 65percent of the female and 35percent of the male population since the literature revealed that males are more at risk for substance use disorder than females.

What is the most appropriate sampling technique in this case?

- A. Simple random sampling
- B. Systematic random sampling
- C. Multistage random sampling
- D. Stratified random sampling

ANSWER: D

175. After obtaining ethical clearance from a university and a support letter from the respective district, a researcher evaluated the effect of group therapy on post-traumatic stress disorder in those who were residing in a migration camp. During the data collection, the researcher created awareness about the study and promised to provide benefits more than they earn and deserve for their participation.

Which ethical principle is most likely violated in this case?

- A. Beneficence
- B. Respect for persons
- C. Non-maleficence
- D. Justice

ANSWER: B

176. A researcher wants to determine medication adherence among epileptic patients on medication. The researcher aims to improve the quality of data by taking care of the sampling procedure, interview technique, and what to do if respondents are not available or refuse to cooperate.

What is the most appropriate quality assurance technique to be used in this study?

- A. Prepare a fieldwork manual for the research team
- B. Train research assistants carefully in all topics covered
- C. Pre-test research instruments and research procedures
- D. Arrange for ongoing supervision of data collections

ANSWER: A

177. An investigator intends to assess the effect of cigarette smoking during pregnancy on birth outcomes among women attending an antenatal care clinic at a hospital. Accordingly, the investigator plans to enrol 120 and 240 pregnant women with and without cigarette smoking, respectively. The possible adverse outcomes to be measured are low birth weight, stillbirth, and pre-term birth. What is the most likely independent variable in this study?

- A. Low birth weight
- B. Cigarette smoking
- C. Effect of cigarette smoking
- D. Pre-term birth

ANSWER: B

178. A researcher conducted a study on major depressive disorder and its risk factors among adults living in a district. The researcher included 2800 study participants randomly and collected data via a face-to-face interview technique using a valid and reliable data collection tool. During the data analysis, major depressive disorders were classified as mild, moderate, and severe.

What is the most appropriate scale of measurement in this scenario?

- A. Nominal
- B. Ordinal
- C. Interval
- D. Ratio

ANSWER: B

179. A researcher determined the systolic blood pressure among individuals with insomnia disorder attending a hospital. Accordingly, the researcher enrolled 456 participants and measured their blood pressure in 10 participants using an aneroid sphygmomanometer early in the morning, and their measurements in mmHg were: 124, 170, 150, 120, 130, 110, 146, 124, 160, and 130.

What is the mean systolic blood pressure of the patients in this scenario?

- A. 130.2 mmHg
- B. 136.4 mmHg
- C. 145.1 mmHg
- D. 142.3 mmHg

ANSWER: B

180. A Zone health department conducted community-based hypertension screening, aiming to plan preventive healthcare services for the community in a district. The department included 2300 adults who were living in the district and collected data on socio-demographic, behavioural risk factors, clinically related factors, and measured blood pressure. Of the total participants, 512 were found to have hypertension.

What is the most appropriate measure of morbidity in this case?

- A. Incidence
- B. Attack rate
- C. Proportion
- D. Prevalence

ANSWER: D

181. A department of psychiatry planned to revise the curriculum that had been used for the last four years. The department head made a team of psychiatry nurse instructors to revise the curriculum and arrange all the facilities for 10 days. The instructors went through all the courses and made modifications for some of the courses. They added two courses because of the advancement of knowledge and practice in the current situation.

What is the most appropriate component of curriculum development in this case?

- A. Research and Technology
- B. Need of a country
- C. Professional environment
- D. Personal interest

ANSWER: A

182. A national expert noticed an unusual rise trend of a disease in recent years, while analysing reports from different regions of the country. The experts held meetings with other stakeholders and decided to include this disease in the surveillance system.

What is the most likely criterion that the experts used in this case?

- A. Severity
- B. Mortality
- C. Prevalence
- D. Preventability

ANSWER: C

183. Zonal health bureau experts conducted a performance review meeting with health information system management (HMIS) experts from the districts. In the meeting, the majority of district participants reported that there was a problem of late reporting of data from lower-level health facilities due to poor infrastructure and frequent electric power interruptions. This caused a delay in reports that were sent from the districts to the zone.

What basic attribute of a good surveillance system is lacking in this case?

- A. Timelines
- B. Accuracy of data
- C. Comprehensive action
- D. Combination of surveillances

ANSWER: A

184. A regional health bureau, in collaboration with other stakeholders, conducted an assessment to identify risk factors for mental illness among high school students. The study result indicated that the majority of the students engaged in tobacco use, alcohol abuse and were physically inactive.

What is the most likely risk factor explained in this scenario?

- A. Modifiable
- B. Intermediate
- C. Environmental
- D. Non-Modifiable

ANSWER: A

185. A city health department conducted a screening to identify the health condition of the resident. During their implementation, the health professionals conducted blood pressure examination, breast cancer testing and eye examination in a mobile clinic.

What is the most appropriate type of screening explained in this case?

- A. Mass screening
- B. Targeted screening
- C. Multiple screening
- D. Opportunistic screening

ANSWER: C

186. In a rural community, health-seeking behaviour for mental health conditions is inadequate. The community prefers spiritual and traditional remedies for such illnesses rather than health facilities. The main reason is due to societal perception that mental illness is a result of a curse, and modern treatments do not have good outcomes.

What is the most likely factor of health behaviour explained in this case?

- A. Norm
- B. Belief
- C. Attitude
- D. Knowledge

ANSWER: B

187. District health office experts have planned to provide community health education on a particular mental health illness. They have an objective to create awareness and promote knowledge with respect to a specific mental health condition.

What is the most likely mental health intervention component to be applied in this case?

- A. Meetings
- B. Public campaign
- C. Social marketing
- D. Social mobilisation

ANSWER: B

188. An assessment was done in collaboration with a regional health bureau and education bureau. The findings showed a high prevalence of substance abuse among adolescents and youths in the schools. To prevent and control substance abuse among school adolescents and youths, the education bureau introduced a new rule in the schools.

What is the level of disease prevention explained in this case?

- A. Tertiary
- B. Primary
- C. Primordial
- D. Secondary

ANSWER: B

189. In a rural village, an adolescent girl who experienced a rape came to a health post with a fear of unwanted pregnancy. After the situation was understood, a Health extension worker and the girl discussed possible solutions and actions to manage the unwanted pregnancy and reach a decision. What is the most likely intervention explained in this case?
- A. Advising
 - B. Counselling
 - C. Health promotion
 - D. Health information
- ANSWER: B
190. District health office experts are working on designing an evaluation of a mental illness intervention that was implemented by health extension workers. As part of the evaluation process, the experts set a timeline for their activity. What is the most likely next step in this case scenario?
- A. Creating a Gantt chart
 - B. Selecting reliable indicators
 - C. Establishing objectives to evaluate
 - D. Determining the appropriate type of evaluation
- ANSWER: C
191. A 26-year-old male patient came to a psychiatry OPD with complaints of low energy, hopelessness, and difficulty in falling asleep. A psychiatric nurse discussed with the patient about drug choices and prescribed appropriate medication for his problem. The nurse informed the patient about the importance of the medication to alleviate the conditions. But the patient denied taking it. What is the likely ethical principle described in this case?
- A. Mal-beneficence
 - B. Autonomy
 - C. Beneficence
 - D. Justice
- ANSWER: B
192. A psychiatry nurse received a patient for admission from the outpatient department. The patient had contact with a duty psychiatry nurse upon admission and onwards. What is the most likely next role of the nurse in this case?
- A. Prescribe a medication
 - B. Order laboratory investigation
 - C. Provide psychotherapy
 - D. Assess the patient's condition
- ANSWER: D
193. A Psychiatry nurse working in a male inpatient ward provided care for the admitted patients with anxiety and underlying medical disorders. Unfortunately, two of the patients needed oxygen support at the same time. The nurse found only one oxygen concentrator in the ward. He confirmed that the dilemma occurred. What is the most likely next step of action to solve the dilemma?
- A. Negotiation
 - B. Examining own value
 - C. Verbalise the problem
 - D. Review of scientific data
- ANSWER: B

194. A suicidal attempt was very common among youths in a town where a psychiatric nurse was living. He recognised that most youths are alcohol and substance abusers. The figure is increasing from time to time. The nurse worried about the situation and planned to tackle this prevalent problem. What is the most appropriate standard of practice that the nurse should follow?

- A. Nursing research
- B. Quality of nursing practice
- C. Nursing education
- D. Assessment of mental health

ANSWER: C

195. A 32-year-old female patient came to a general hospital with a complaint of difficulty in falling asleep and was diagnosed with insomnia. She had been previously treated for a similar case with a different psychiatrist. Concerning recurrent attachment and prescription of the same medication frequently, the patient wanted more time to discuss with the current psychiatrist. The psychiatrist was not interested, but reluctantly gave her time to recheck her conditions.

What is the most likely element of compassionate care missed in this case?

- A. Virtue
- B. Relational space
- C. Virtuous response
- D. Seeking to understand

ANSWER: D

196. A 45-year-old male patient came to the Psychiatry OPD with a diagnosis of personality disorder. A psychiatric nurse created a good rapport and discussed with the patient about possible treatment options, and let the patient choose from among the options.

What is the most likely patient right implemented in this case?

- A. The right to the truth
- B. The right to be kept private
- C. The right to be informed
- D. The right to refuse the care

ANSWER: C

197. A Psychiatry nurse working in an inpatient department knew about all personal details of the patients and the history of their families during a therapeutic interpersonal relationship. The nurse shared their sensitive information with others, which jeopardised the social life of the patients.

What is the most likely ethical issue affected by the nurse in this scenario?

- A. Privacy
- B. Autonomy
- C. Confidentiality
- D. Freedom of information

ANSWER: C

198. A 20-year-old male patient was admitted to a psychiatry ward with Bipolar disorder. He became agitated and was speaking abusive terms. A psychiatric nurse who was caring for the patient administered sedative medication without the order of a physician and without informing the patient. The patient experienced adverse drug effects.

What is the most likely psychiatric nursing malpractice in this case?

- A. The occurrence of drug adverse effects
- B. The act of giving medication without consent
- C. The act of giving medication to calm patients
- D. The act of using medication as a first line before physical restraining of patients

ANSWER: B

199. A 69-year-old conscious and capable patient is expressing his willingness to give authorised legal authority to his wife to decide the next plan of care with a psychiatry nurse. The nurse is searching for the best recording form to hold documentation of the patient's interests.

What is the most likely documentation form to be used in this case?

- A. Living will
- B. Advance directive
- C. Do not resuscitate (DNR)
- D. Durable power of attorney

ANSWER: B

200. A psychiatry nurse working in a psychiatry ward encountered a patient who came from a rural area with an anxiety disorder. The patient had many concerns to discuss with a psychiatrist, but she was unable to do so. The nurse recognised the patient's problem and decided to explain her concerns to the psychiatrist. But, the nurse perceived herself as not being loyal to the institution and cutting the working hours.

What is the most likely barrier to advocacy that the psychiatry nurse experienced?

- A. Short staffing
- B. Knowledge deficits
- C. Power differentials
- D. Fatigue

ANSWER: C